

JOHNS HOPKINS HOSPITAL – NEUROLOGY REFERRAL FORM (FAX RECEIVED)

Date: 04/21/2026 Time: 16:42

Fax #: (410) 955-xxxx

Referring Office: NOVA Family Practice / Urgent Care Hybrid

Reason for Referral:

r/o stroke vs mass vs demyelinating? new weakness + speech issue

Urgency: Routine (checked) Urgent (handwritten circle?)

Requested Service: Neurology (general) / Stroke clinic? (illegible overwrite)

Referring Provider:

Arjun Patel, MD

Phone: 703-555-8821

Fax: 703-555-8830

Patient Name: HENDERSON, MARIA L

DOB: 03/14/1972

Insurance: CareFirst BCBS (copy attached maybe)

Notes:

pt with progressive L sided symptoms x ~2-3 weeks, initially thought possible TIA vs migraine variant but now persistent. imaging done outside. pls eval.

Attachments: yes (18 pages total? received 16)

Page 1 of 16

PATIENT DEMOGRAPHICS (AUTO-GENERATED)

Name: Maria Louise Henderson

MRN: 8892341 (NOVA FP)

DOB: 03/14/1972

Age: 54

Sex: F

Address: 4821 Maple Ridge Dr, Centreville VA 20121

Phone: 703-555-2219 (home) / 571-555-9920 (cell)

Emergency Contact:

Henderson, Paul (spouse) 703-555-2201

Primary Insurance: CareFirst BCBS PPO

Secondary: none listed

Employer: Fairfax County Public Schools (teacher)

Allergies: NKDA (one note says “penicillin—rash?” unclear)

Preferred Pharmacy: CVS #17821 Centreville

Language: English

Smoking Status: Former smoker (quit 2015) vs “never smoker” in older note

Alcohol: occasional wine 2–3/week

Height: 5’6” Weight: 168 lb (last recorded)

Page 2 of 16

REFERRING PROVIDER NOTE (04/20/2026)

Provider: A. Patel, MD

Chief Complaint: “left arm not working right and slurring words sometimes”

HPI:

54F w hx HTN, migraines, hypothyroidism presents for eval of ongoing neurologic symptoms. approx 3 wks ago pt noted intermittent L hand clumsiness (dropping pen, difficulty typing). initially episodic lasting minutes, now more persistent. over past 5–7 days family notes mild slurred speech and occasional word-finding difficulty. pt denies facial droop but husband thought “mouth looked uneven once.”

pt seen in urgent care 04/05 for similar → diagnosed probable complex migraine vs TIA. started on ASA.

no acute severe headache. no LOC. no seizure activity witnessed. pt does report intermittent dull R-sided headache “behind eye” not typical of her migraines.

ROS:

- weakness LUE
- speech changes
- vision loss (though one note mentions “blurry” unclear)
- fever
- recent infection

Exam:

BP 148/92

Neuro: mild L pronator drift. strength 4+/5 LUE, 5/5 elsewhere. subtle dysarthria. gait intact.

Assessment:

subacute focal neurologic deficit unclear etiology

consider:

- ischemic event (subacute)
- demyelinating process
- atypical migraine
- less likely mass? (but progressive)

Plan:

MRI brain w/wo contrast ordered (done 04/18 outside facility)

refer neurology for further eval

continue ASA 81 mg

NOTE: MRI report attached (see later pages)

Page 3 of 16

PRIOR NOTE #1 (URGENT CARE VISIT 04/05/2026)

Facility: NOVA Urgent Care

Provider: L. Simmons, PA-C

CC: “left hand numb and headache”

HPI:

pt reports sudden onset L hand tingling while grading papers. lasted ~10 min, resolved. associated mild headache R side. hx migraines. denies weakness at that time. no speech changes per pt.

Exam:

normal neuro exam. no deficits appreciated.

Assessment:

likely migraine aura vs TIA

Plan:

ASA 81 mg daily

outpatient PCP follow up

ER if symptoms recur

(addendum handwritten: “pt returned call next day still mild clumsiness?”)

Page 4 of 16

PRIOR NOTE #2 (PRIMARY CARE VISIT 04/10/2026)

Provider: Dr. Patel

CC: follow-up urgent care

HPI:

pt states symptoms not fully resolved. now reports dropping objects occasionally. denies speech difficulty (contradicts later). headache persistent but mild.

PMH:

HTN

Hypothyroid

Migraine w aura

GERD

“anxiety?”

Medications reviewed (see med list)

Exam:

neuro grossly normal except “maybe subtle decreased grip L vs R”

Assessment:

unclear etiology. possible prolonged migraine vs cervical radiculopathy?

Plan:

trial NSAIDs

consider imaging if persists

Page 5 of 16

PRIOR NOTE #3 (ED VISIT 04/17/2026 – OUTSIDE HOSPITAL)

Facility: Fair Oaks Hospital

Arrival Time: 13:12

Chief Complaint: slurred speech

HPI:

pt presents after husband noted slurred speech this morning. pt states symptoms ongoing ~2 weeks but worse today. denies headache at time of arrival. denies trauma.

Exam:

NIHSS 2 (mild dysarthria, L arm drift)

CT Head (non-contrast): no acute hemorrhage. "subtle area low attenuation R frontal? artifact vs early ischemia"

Labs: unremarkable

Disposition:

not candidate for thrombolytics (symptom onset unclear)

advised MRI outpatient

discharged home

Discharge dx: TIA vs evolving stroke

Page 6 of 16

LAB RESULTS (MULTIPLE DATES MIXED)

CBC (04/17):

WBC 6.8

Hgb 13.2

Plt 248

BMP (04/17):

Na 139

K 4.2

Cr 0.9

Glucose 102

TSH (03/01): 3.8

Lipid panel (02/2026):

LDL 142 (elevated)

HDL 48

A1c (02/2026): 5.6

Vitamin B12 (04/10): 410

ESR (04/20): 18

CRP (04/20): 2.1 (slightly high?)

Urinalysis (irrelevant?): normal

Page 7 of 16

IMAGING REPORT (MRI BRAIN W/WO CONTRAST – 04/18/2026)

Facility: Fairfax Imaging Center

Indication: focal neurologic deficit

Technique: MRI brain with and without gadolinium

Findings:

There is a 2.8 x 2.4 cm lesion in the right frontal lobe, subcortical white matter extending toward precentral gyrus. lesion demonstrates heterogeneous enhancement with surrounding vasogenic edema. mild mass effect with effacement of adjacent sulci. no midline shift.

Diffusion: no restricted diffusion to suggest acute infarct.

GRE: no hemorrhage.

Impression:

Enhancing right frontal lesion with surrounding edema. differential includes primary neoplasm (e.g., glioma), metastasis, less likely tumefactive demyelination. clinical correlation recommended.

(typed note at bottom: “results phoned to ordering provider 04/18 18:22”)

Page 8 of 16

CT HEAD REPORT (FROM ED 04/17 – REPEATED)

Impression:

No acute intracranial hemorrhage. possible subtle hypodensity right frontal region, recommend MRI correlation.

Page 9 of 16

MEDICATION LIST (AUTO-EXTRACTED, NOT VERIFIED)

Levothyroxine 75 mcg daily
Lisinopril 10 mg daily
Omeprazole 20 mg daily
Aspirin 81 mg daily (started recently)
Ibuprofen 400 mg PRN
Sumatriptan 50 mg PRN migraine
Vitamin D 1000 IU daily
Multivitamin

Fluticasone nasal spray
Cyclobenzaprine (old Rx, still listed)
Sertraline 25 mg daily (pt unsure if taking)

Page 10 of 16

DISCHARGE SUMMARY (ED – 04/17/2026)

Patient: Maria Henderson
MRN: FOH-55621

Summary:
54F evaluated for possible TIA/stroke. CT negative. mild deficits persisted but stable.
recommended outpatient MRI and neurology follow-up.

Instructions:
return if worsening weakness, speech changes, headache

Medications: continue home meds, ASA

Page 11 of 16

PROCEDURE NOTE (UNRELATED – COLONOSCOPY 2023)

Indication: screening

Findings:
two small polyps removed

Recommendation: repeat 5 yrs

(appears auto-included in packet)

Page 12 of 16

PRIMARY CARE NOTE (OLDER – 2025)

HPI:
pt well, migraines controlled

(no neuro deficits documented)

Page 13 of 16

FAX COVER (FROM IMAGING CENTER)

To: Dr Patel
Re: Henderson MRI

“abnormal study pls review”

Page 14 of 16

HANDWRITTEN NOTE (SCANNED, HARD TO READ)

“pt worsening? speech more noticeable per husband
consider sooner neuro?
maybe steroid? call?”

Page 15 of 16

INSURANCE CARD COPY (BLURRY)

Name: Henderson, Maria

Page 16 of 16